SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Aflibercept + FOLFIRI

Metastatic colorectal cancer · NCCP 00238 · version 6 · 14-day cycle

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Patient name	DOB	Hospital no.	Date

Treatment context
Aflibercept with FOLFIRI is used for metastatic colorectal cancer that is resistant to, or has progressed after, an oxaliplatin-containing treatment. The aim is to control the cancer and symptoms; treatment continues while its benefit outweighs its side effects.

Schedule
Day 1: aflibercept, then irinotecan, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. Repeat every 14 days while beneficial and tolerable.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Aflibercept
ANTI-ANGIOGENIC FUSION PROTEIN

- ☐ High blood pressure / urine protein
- ☐ Bleeding and blood clots
- ☐ Delayed wound healing
- ☐ Rare bowel perforation / fistula
- ☐ Rare serious neurological effects
- ☐ Infusion reaction

Irinotecan
TOPOISOMERASE I CHEMOTHERAPY

- ☐ Early cramps, sweating and diarrhoea
- ☐ Delayed diarrhoea and dehydration
- ☐ Low blood counts / serious infection
- ☐ Nausea, mouth soreness and fatigue
- ☐ Hair thinning or loss
- ☐ Rare serious lung or allergic reactions

Folinic acid
CHEMOTHERAPY MODULATOR

- ☐ Enhances fluorouracil activity and toxicity
- ☐ Rare allergic reaction
- ☐ Diarrhoea / mouth soreness in combination

Fluorouracil
CHEMOTHERAPY

- ☐ Diarrhoea, dehydration and mouth ulcers
- ☐ Low blood counts / infection
- ☐ Hand-foot skin soreness
- ☐ Cardiotoxicity including chest pain / ischaemia
- ☐ Severe early toxicity including DPD-related toxicity
- ☐ Pump / central-line complications

Confirm the individual aim, likely benefit and alternatives, DPD result, the 46-hour pump, disconnection and urgent contacts. Explain early irinotecan symptoms separately from delayed diarrhoea. Explain blood-pressure/urine checks, bleeding/clot risks and procedure/wound precautions.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00238 v6.

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